Massachusetts Department of Public Health Standing Order 2026 - 06
Standing Order for Dispensing and Administration of Influenza Vaccine: 2026-2027

This standing order authorizes qualified pharmacy personnel pursuant to 105 CMR 700.004(B)(6) to dispense and administer Influenza vaccine in Massachusetts to all eligible persons as described below.

PROCEDURE

1) Assess for need of vaccination against influenza
    a. For Children and Adolescents
        - All people 6 months of age and older are recommended to receive influenza vaccination each year unless there is a contraindication to vaccination.
        - A second dose of influenza vaccine is recommended 4 weeks or more after the first dose for children age 6 months through 8 years if they have not or don't know if they have received 2 doses in prior years (not necessarily in the same season).
        - A second dose is needed for a 9-year-old child who received one dose in the current season when they were age 8 years, if they have not or don't know if they have received 2 doses in prior years.
        - Inactivated influenza vaccine (IIV) or recombinant influenza vaccine (RIV) may be administered at any time before, after, or simultaneously with other recommended vaccines. Live attenuated influenza vaccine (LAIV) may be administered without regard to timing of non-live vaccines but should be administered on the same day or at least 4 weeks apart from another live virus injectable vaccine.
    b. For Adults
        - All adults are recommended to receive influenza vaccination each year unless there is a contraindication to vaccination.
        - Adults age 65 years and older should preferentially receive any one of the following higher dose or adjuvanted influenza vaccines: high-dose inactivated influenza vaccine (HD-IIV), recombinant influenza vaccine (RIV), or adjuvanted IIV (aIIV). If none of these three vaccines are available, then any other age-appropriate influenza vaccine should be used.
        - Solid organ transplant recipients (SOTR) age 18 through 64 years who are on immunosuppressive medication regimens may receive HD-IIV or aIIV influenza vaccine as acceptable options for influenza vaccination, without a preference over other age-appropriate IIVs or RIVs.
        - Adults who are or will be pregnant during the influenza season: administer any recommended, age-appropriate IIV or RIV to pregnant people in any trimester.
        - Adults who do not recall whether they received influenza vaccine in the current season should be vaccinated.
        - Adults and children who recently received another vaccine, including COVID-19 vaccine, may be administered IIV or RIV at any time before, after, or simultaneously (on the same day, at separate anatomic sites). Live attenuated influenza vaccine (LAIV) may be administered without regard to timing of non-live vaccines but should be administered on the same day or at least 4 weeks apart from an injectable live virus vaccine. Information on coadministration of all vaccines can be found at www.cdc.gov/vaccines/hcp/imz-best-practices/timing-spacing-immunobiologics.html and information on giving 2 or more intramuscular vaccines can be found at www.immunize.org/catg.d/p2030.pdf

2) Screen for contraindications and precautions.

   a. Contraindications:
      a. History of anaphylaxis or a severe allergic reaction after a previous dose of the influenza vaccine or to a component of the influenza vaccine.
      b. Do not give LAIV to any person who
         - Is pregnant.
         - Is age 50 years or older.
         - Is age 2-4 who has received a diagnosis of asthma or who has experienced wheezing or asthma within the past 12 months, based on a healthcare provider's statement or medical record.
         - Is age 6 months through 17 years and is receiving aspirin or salicylate-containing medicine.
         - Has functional or anatomic asplenia, cochlear implant, or is immunocompromised due to any condition (including due to medications or underlying diagnosis such as HIV).
         - Has active communication between cerebrospinal fluid (CSF) and the oropharynx, nose, or ear or any other cranial CSF leak.
         - Received influenza antivirals before scheduled vaccination (zanamivir or oseltamivir within 48 hours; peramivir within 5 days; baloxavir within 17 days). If any of these antiviral drugs are taken within 14 days after LAIV, revaccinate with IIV or RIV.
         - Is a close contact for a severely immunosuppressed person who requires a protected environment
   b. Precautions:
      i. A diagnosed non-severe allergy to a component of the influenza vaccine (other than egg).
      ii. Non-severe, immediate (onset less than 4 hours) allergic reaction after administration of a previous dose of one influenza vaccine type, if receiving the same vaccine type.
      iii. Moderate to severe acute illness, with or without fever.
      iv. History of Guillan-Barre syndrome within 6 weeks of previous influenza vaccination.
      v. For LAIV only:
         - Asthma in a person 5 years of age or older
         - Other medical conditions that would predispose a person to complications (e.g. other chronic pulmonary, cardiovascular, renal, hepatic, hematologic, and neurologic disorders with the exception of hypertension).

   c. Not a contraindication or precaution:
      Egg allergy of any severity is not considered to be a contraindication or a precaution to administration of any influenza vaccine (egg-based or non-egg-based). People with any type of egg allergy may receive any IIV, RIV, or live attenuated influenza vaccine (LAIV) that is otherwise appropriate for their age and health status. Safety measures beyond those recommended for receipt of any vaccine are not recommended.

3) Provide Vaccine Information Statements (VIS).

4) Prepare to administer vaccine.
   a. Verify that the label on the vaccine states 2026-2027 Formula.
   b. For vaccine that is to be administered intramuscularly, choose the needle gauge, needle length, and injection site.
   c. For LAIV, which is administered intranasally, prepare the vaccine according to directions in the package insert

5) Administer influenza vaccine per procedure outlined below.

For intranasal administration

- FluMist (LAIV3[i]) single use nasal spray, 2026-2027 Formula
  Dispense single 0.2 mL dose for:
  - Individuals 2 through 49 years
    Directions for use: Administer 0.1 mL to each nostril (0.2 mL total volume)

For intramuscular administration

- Fluarix (IIV3[ii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 6 months of age and older
    Directions for use: Administer Fluarix as a single 0.5 mL dose

  OR

- FluLaval (IIV3[ii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 6 months of age and older
    Directions for use: Administer FluLaval as a single 0.5 mL dose

  OR

- Flublok (RIV3[iii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 9 years of age and older
    Directions for use: Administer Flublok as a single 0.5 mL dose

  OR

- Fluzone (IIV3[ii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 6 months of age and older
    Directions for use: Administer Fluzone as a single 0.5 mL dose

  OR

- Fluzone (IIV3[ii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in single dose vial for:
  - Individuals 6 months of age and older
    Directions for use: Administer Fluzone as a single 0.5 mL dose

  OR

- Fluzone (IIV3[ii]) multi-dose injectable suspension 5.0mL multidose vial, 2026-2027 Formula
  Dispense 0.25 ml or 0.5 ml
  - Individuals 6 through 35 months

Directions for use: Administer Fluzone as a single 0.25 mL dose or as a single 0.5 mL dose
- Individuals 3 years of age and older
  Directions for use: Administer Fluzone as a single 0.5mL dose

OR

- Fluzone High-Dose (HD-IIV3[ii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 65 years of age and older
    Directions for use: Administer Fluzone High-Dose as a single 0.5 mL dose
  - Solid organ transplant recipients 18-64 years of age
    Directions for use: Administer Fluzone High-Dose as a single 0.5 mL dose

OR

- Afluria (IIV3[ii]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 3 years of age and older
    Directions for use: Administer Afluria as a single 0.5 mL dose

OR

- Afluria (IIV3[ii]) multi-dose injectable suspension 5.0mL multidose vial, 2026-2027 Formula
  Dispense 0.25 ml or 0.5 mL from multi-dose vial for:
  - Individuals 6 through 35 months
    Directions for use: Administer Afluria as a single 0.25 mL dose
  - Individuals 3 years of age and older
    Directions for use: Administer Afluria as a single 0.5mL dose

OR

- Fluad (aIIV3[iv]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5mL dose supplied in prefilled syringes for:
  - Individuals 65 years of age and older
    Directions for use: Administer Fluad as a single 0.5 mL dose
  - Solid organ transplant recipients aged 18-64
    Directions for use: Administer Fluzone High-Dose as a single 0.5 mL dose

OR

- Flucelvax (ccIIV3[v]) single dose injectable suspension, 2026-2027 Formula
  Dispense single 0.5 mL dose supplied in prefilled syringes for:
  - Individuals 6 months of age and older
    Directions for use: Administer Flucelvax as a single 0.5 mL dose

OR

- mFlusivia (mRNA)
  Dispense 0.38 ml prefilled syringe for:

- Individuals 50 years of age and older
  Directions for use: Administer mFlusivia as a single 0.38 mL dose

6) Document the vaccination.

7) Be prepared to manage medical emergencies.

8) Report All Adverse Events to VAERS. Report all adverse events following the administration of influenza vaccine to the federal Vaccine Adverse Event Reporting System (VAERS). To submit a VAERS report online (preferred) or to download a writable PDF form, go to https://vaers.hhs.gov/reportevent.html. Further assistance is available at (800) 822-7967.



September 30, 2026

Physician's Signature *Date*

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